Market Access Insight: Keytrava in Germany (2023)

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Access Overview

Reimbursement for Keytrava in Germany was approved on a managed entry agreement requiring real-world evidence submission within 12 months of launch under BfArM.

The statutory health insurance system applies health technology assessment before listing Keytrava for non-small cell lung cancer.

Payer Perspectives

Payers in Germany view the price of Keytrava as acceptable only with a demonstrated clinical benefit versus the standard of care.

Formulary placement is reviewed annually, and pricing is linked to outcomes in the registry.

Supply and logistics are not a concern for Keytrava, though cold-chain handling and nurse-led administration add workload in busy clinics.

Respondents expect real-world persistence on Keytrava to be lower than in trials, mainly because of treatment burden over long durations and the complexity of managing pneumonitis.

Medical education priorities include recognising fatigue early, communicating realistic expectations, and coordinating with primary care.

Subgroup analyses suggest that benefit with Keytrava is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Guideline committees are expected to review the evidence base for non-small cell lung cancer again next year, which could change eligibility criteria and referral patterns.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Keytrava has no direct substitute.

Peer-to-peer education and congress data were cited as the most trusted sources when oncologists evaluate new evidence on PD-1 inhibitor agents.

Evidence Requirements

Required evidence includes overall survival or response rate data, patient-reported outcomes and comparative effectiveness against Keytrava's main competitor.

Cost-effectiveness submissions typically model first-line use, and payers question the durability of benefit beyond the trial follow-up period.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Keytrava and competing agents.

Respondents expect real-world persistence on Keytrava to be lower than in trials, mainly because of missed maintenance doses and the complexity of managing rash.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Oncologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between PD-1 inhibitor options.

Registry data show that outcomes in non-small cell lung cancer depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

Clinical guidelines in Germany recommend first-line use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Pricing and Reimbursement Outlook

A confidential rebate is expected to remain in place until 2025. Market access risk is high for new indications.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

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Supply and logistics are not a concern for Keytrava, though cold-chain handling and nurse-led administration add workload in busy clinics.

The unmet need in non-small cell lung cancer persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Clinical guidelines in Germany recommend refractory disease use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

Cost-effectiveness submissions typically model refractory disease use, and payers question the durability of benefit beyond the trial follow-up period.

Safety monitoring focused on infusion reactions and diarrhoea, with dose interruption or discontinuation used for severe adverse events.

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